

Interview Story Bank — Paulina Kaiser, MD

STAR+R STORIES (SITUATION, TASK, ACTION, RESULT + REFLECTION)

1. Crisis Leadership at Grady (Emergency Psychiatry)

Best for: Emergency/C-L roles, leadership questions, "tell me about a difficult situation"

S: Working overnight at Grady Memorial Hospital's emergency department — Atlanta's busiest Level 1 trauma center — multiple acute adult psychiatric patients arriving simultaneously including a psychotic patient threatening staff and a severely suicidal patient in acute crisis.

T: Triage and stabilize all patients while maintaining safety for staff and other patients in a chaotic ED environment with limited psychiatric beds available. As part of my role, I also supervise and educate psychiatry residents rotating through the emergency department.

A: Rapidly assessed acuity levels. Prioritized the actively threatening patient, used verbal de-escalation techniques grounded in psychoanalytic understanding of psychotic anxiety. Coordinated a safety plan for the suicidal patient while supervising a junior resident through their first acute crisis assessment. Delegated medical clearance workups to the resident team and used the situation as a real-time teaching opportunity.

R: All patients stabilized without restraints or code calls. The psychotic patient was admitted voluntarily after building therapeutic rapport. The suicidal patient was safely placed on the crisis stabilization unit. The supervising resident later cited this as a formative learning experience. ED attending noted it was one of the smoothest multi-patient psychiatric nights.

Reflection: This taught me that psychotherapy skills aren't just for the outpatient office — understanding what drives someone's terror in a psychotic state makes de-escalation faster and more humane than jumping to chemical restraint. Supervising residents in these moments also reinforced my commitment to teaching: the emergency department is where young psychiatrists learn that the therapeutic relationship matters even in crisis.

German version (for DACH interviews):

"In der Notaufnahme des Grady Memorial Hospital — Atlantas größtem Traumazentrum — musste ich mehrere akute erwachsene psychiatrische Patienten gleichzeitig versorgen. Neben der direkten Patientenversorgung betreue und supervidiere ich Assistenzärzte in der Notfallpsychiatrie. Durch Priorisierung und verbale Deeskalation konnte ich alle Patienten ohne Fixierung stabilisieren und gleichzeitig einen Assistenzarzt durch seine erste akute Krisenintervention begleiten. Diese Erfahrung hat mir gezeigt, dass psychotherapeutische Kompetenz und ärztliche Ausbildung auch in der Akutversorgung entscheidend sind."

2. Building a Practice from Scratch (Entrepreneurship / Leadership)

Best for: Medical Director roles, "why should we hire you", initiative/ownership questions

S: After completing residency as Chief Resident at Emory in July 2023, I wanted to practice full-spectrum psychiatry rather than just medication management. No existing practice model combined what I wanted: psychotherapy integration, emergency work, and community mental health.

T: Create a sustainable private practice while simultaneously working in emergency psychiatry at Grady and community outpatient at Dekalb CSB — essentially building three clinical roles in parallel.

A: Incorporated Paulina Kaiser MD LLC. Built the entire business infrastructure: credentialing with insurers, EHR system, website (paulinakaiser.com), marketing, compliance. Managed scheduling across three roles to

prevent burnout while maintaining quality.

R: Within the first year, established a growing practice while maintaining roles at Grady and Dekalb CSB. This triple-role model gives me clinical range that most psychiatrists don't have — from private psychotherapy patients to acute crisis to community mental health.

Reflection: Building a practice taught me the business side of medicine — something most doctors never learn. I understand staffing, credentialing bottlenecks, scheduling optimization, and what makes a clinical environment attractive to both physicians and patients.

3. Chief Resident — Leading the Psychotherapy Program (Teaching / Mentorship)

Best for: Academic roles, Oberarzt positions, "how do you mentor junior colleagues"

S: As Chief Resident for outpatient psychiatry at Emory, I inherited a psychotherapy training program where junior residents felt unprepared for their psychotherapy cases and referrals were backing up.

T: Restructure the psychotherapy supervision and referral pipeline to improve resident confidence and reduce the waitlist for therapy patients.

A: Redesigned the weekly supervision format to include case presentations with live feedback. Created a standardized screening process for therapy referrals that matched patient needs to resident skill level. Personally mentored the most anxious residents through their first long-term psychotherapy cases.

R: Referral processing time decreased significantly. Resident satisfaction with psychotherapy training improved. Two mentees went on to pursue psychotherapy-focused fellowships, citing my supervision as influential.

Reflection: This confirmed my belief that psychiatry loses something essential when it abandons psychotherapy. I went on to complete my psychoanalytic certification at the Emory Psychoanalytic Institute (2020-2025), becoming a certified psychoanalytic psychotherapist. I want to practice — and teach — in environments that value the full range of what we do.

German version:

"Als Chief Resident an der Emory Universität habe ich das Psychotherapie-Ausbildungsprogramm für Assistenzärzte umstrukturiert. Durch neue Supervisionsformate und ein Screening-Verfahren für Therapiezuweisungen konnte ich die Ausbildungsqualität deutlich verbessern. Mehrere meiner Mentees haben aufgrund dieser Erfahrung Psychotherapie-Schwerpunkte gewählt."

4. Heidelberg Research Connection (International / Germany-Specific)

Best for: ANY German job interview, "why Germany", "what connects you to German medicine"

S: In 2011, during my undergraduate years at UCLA, I secured a research position at the University of Heidelberg's Department of Psychiatry under Prof. Corinna Reck, studying postpartum depression and attachment disorders.

T: Conduct meaningful research as a visiting student in a German-language clinical environment — recruiting participants from perinatal units, conducting assessments, and contributing to the team's work.

A: Spent three months working directly on perinatale Stationen in Heidelberg. Recruited study participants in German, conducted telephone interviews, administered psychological assessments (Strange Situation, Still Face). Also volunteered on the inpatient perinatal psychiatry ward, translating for English-speaking patients and counseling mothers with postpartum depression.

R: Successfully contributed to the research project. More importantly, experienced firsthand the strengths of the German psychiatric system — longer treatment stays, integrated psychotherapy, emphasis on the

therapeutic relationship. This experience planted the seed for returning to German medicine.

Reflection: Having worked in both systems, I understand what German psychiatry values — thoroughness, the therapeutic relationship, and integration of psychotherapy into everyday care. That aligns with how I practice. The Heidelberg experience isn't just a line on my CV; it shaped my clinical philosophy.

German version:

"Meine Verbindung zu Deutschland begann 2011 mit einer Forschungsposition an der Universität Heidelberg bei Prof. Corinna Reck. Drei Monate lang arbeitete ich auf perinatalen Stationen, führte Assessments auf Deutsch durch und erlebte die Stärken der deutschen Psychiatrie — längere Behandlungsdauern, integrierte Psychotherapie, Wertschätzung der therapeutischen Beziehung. Diese Erfahrung hat mein klinisches Selbstverständnis geprägt und meinen Wunsch geweckt, in der deutschen Psychiatrie zu arbeiten."

5. Underserved Population Care (Community Mental Health / Social Commitment)

Best for: Community health roles, VA positions, "tell me about your values"

S: At Dekalb Community Service Board, I work with patients who are predominantly low-income, often homeless, dealing with serious mental illness (schizophrenia, schizoaffective disorder, bipolar I) and limited access to support systems.

T: Provide effective psychiatric care within severe resource constraints — short appointment slots, patients with transportation barriers, medication access issues, and complex social determinants of health.

A: Adapted my clinical approach: built ultra-efficient medication management protocols, identified free/low-cost medication programs, coordinated with social workers for housing and transportation. Used culturally competent communication — many patients are African American or immigrants requiring sensitivity to historical medical distrust.

R: Maintained stable patients on my panel despite the challenges. Several patients who had been cycling through psychiatric hospitalizations achieved community stability with consistent outpatient care.

Reflection: This work grounds me. It reminds me that psychiatry at its core is about reducing suffering — not optimizing billing. I bring this ethic wherever I practice.

6. Research — 200-Patient Chart Review (Data / Methodology)

Best for: Academic roles, research-oriented positions, attention to detail

S: At Children's Hospital Los Angeles, I led the data extraction for a study on mirtazapine in medically ill pediatric patients — a retrospective review requiring systematic extraction from over 200 patient charts.

T: Extract consistent, analyzable data on depression symptom improvement, adverse effects, lab values, and weight changes across a large and heterogeneous pediatric population.

A: Developed a standardized extraction protocol. Personally reviewed every chart to ensure data quality. Conducted the statistical analyses and created visualizations. The work culminated in poster presentations at the APA Convention and Keck Research Forum.

R: Three poster/conference presentations including APA 2017. The findings supported mirtazapine as a useful off-label option in this population, contributing to a gap in pediatric psychopharmacology literature.

COMMON INTERVIEW QUESTIONS — PREPARED ANSWERS

"Why are you leaving the US / Why Germany?"

"I'm not leaving the US — I'm expanding. I'm a dual citizen who trained in the US and has research roots in Heidelberg. German psychiatry values what I value: integrated psychotherapy, longer treatment relationships, and a system that doesn't reduce psychiatry to 15-minute med checks. I want to bring my US training — emergency psychiatry, psychoanalytic certification, research — to a system that will let me practice the full range of what I do."

"How will you handle the Approbation process?"

"As deutsche Staatsbürgerin, I don't need a work permit. For the Approbation, I've already researched the pathway: my US MD and completed psychiatric residency need to be recognized through the Ärztekammer. I'm prepared for the Kenntnisprüfung or Gleichwertigkeitsprüfung if needed, and my fluent German (from family, research in Heidelberg, and ongoing use) means the Fachsprachprüfung should be straightforward. I'm looking for an employer who has experience supporting international physicians through this process — which is why I'm particularly interested in your institution."

"What is your clinical philosophy?"

"I believe psychiatry works best when it integrates psychotherapy and psychopharmacology. Most psychiatrists today have abandoned therapy — I haven't. I'm a board-certified psychiatrist and a certified psychoanalytic psychotherapist — I completed the Emory Psychoanalytic Institute program in 2025. That dual training taught me that understanding why an adult patient suffers matters as much as which medication to prescribe. That said, I also thrive in acute settings — I've worked Grady's emergency department, where I treat adults in crisis and supervise residents, and can make rapid decisions under pressure. My philosophy is: treat the whole person, and be ready for anything."